

OREGON STATE HOSPITAL

POLICY

SECTION 6: Patient Care **POLICY:6.069**

SUBJECT: Escalation of Clinical Concerns

POINT PERSON: Chief Medical Officer

APPROVED: James Diegel
Interim Superintendent



DATE:6/13/2025

SELECT ONE: ☒ New policy

☐ Minor/technical revision of existing policy

☐ Reaffirmation of existing policy

☐ Major revision of existing policy

I. PURPOSE AND APPLICABILITY

- A. Issues that indicate the need for immediate clinical intervention shall be promptly communicated to a practitioner responsible for the patient's care. Team members shall promptly escalate unresolved concerns for patient safety using established escalation guidelines in this policy and its attachments.
- B. This policy applies to all staff.

II. POLICY

- A. All hospital employees, contractors and providers are obliged to escalate any concern they have over the safety or well-being of any patient, staff member or visitor, through the chain of command up to and including the executive administrator on call.
- B. Time Frame
 - 1. Escalations of patient, visitor or staff safety concerns should occur within a clinically appropriate time frame.
 - 2. Escalations of critical concerns should occur immediately (within 15 minutes or less).
 - a. A concern is considered "critical" if the concerns may represent a life-threatening situation.
- C. General Chain of Command

1. Safety concerns should be escalated until resolved to the satisfaction of the team member through the following steps (when applicable).
 - a. The Lead Nurse with immediate responsibility for the care of the patient
 - b. A physician or practitioner responsible for the care of the patient
 - c. A Program Nurse Manager (PNM) or Director of Nursing Services (DNS)
 - d. The executive administrator on call
2. The team member with the concern should also inform and seek the assistance of appropriate support and supervisory personnel, such as the lead nurse or manager.

D. Physician Response

1. Physicians are expected to respond (by telephone or in person):
 - a. Within 15 minutes of an emergency or emergent request by a unit staff member and
 - b. Within 30 minutes of any other request.

E. Incident Reporting

An incident report should be completed whenever an attempt to resolve the concern is unsuccessful at any level (e.g. unable to contact the physician for a critical concern within 15 minutes).

- F. OSH follows all applicable regulations, including federal and state statutes and rules; Oregon Department of Administrative Services (DAS), Shared Services, and Oregon Health Authority (OHA) policies; and relevant accreditation standards. Such regulations supersede the provisions of this policy unless this policy is more restrictive.

III. DEFINITIONS

- A. "Staff" includes employees, volunteers, trainees, interns, contractors, vendors, and other state employees assigned to work at Oregon State Hospital (OSH).

IV. PROCEDURES

V. ATTACHMENTS

Attachment A Example of Escalation for a Clinical Concern

VI. RELATED OSH FORMS

VII. RELATED OSH POLICIES

- 6.001 "Patient Transfers"
- 6.003 "Seclusion and Restraints"
- 6.005 "Patient Death and Organ Donation"
- 6.006 "On grounds Off grounds Movement"
- 6.010 "Enhanced Supervision"
- 6.011 "Treatment Care Planning"
- 6.012 "Code Status"
- 6.013 "Discharge and Conditional Release Planning"
- 6.016 "Sexual Activity Involving Patients"
- 6.020 "Interpreter Language Services for Patients"
- 6.024 "Transportation and Supervision Ratios"
- 6.025 "Advance Directives"
- 6.028 "Education for Patients"
- 6.029 "Risk Review and Privileges"
- 6.030 "Electronic Device and Internet Access"
- 6.031 "Dysphagia Screening"
- 6.032 "Marriage Requests"
- 6.035 "Medication Reconciliation"
- 6.042 "Short-Term Assessment of Risk and Treatability (START)"
- 6.044 "Guardianship"
- 6.045 "Clinical Documentation"
- 6.046 "Patient Falls Program"
- 6.047 "Patient Food"
- 6.052 "Trauma Screening"
- 6.053 "Education Services for Patients Under 21"
- 6.054 "Laboratory Critical Values or Results"
- 6.055 "Medication Management"
- 6.056 "Suicide Risk Screening, Assessment"
- 6.058 "Admissions"
- 6.059 "Animals at OSH"
- 6.060 "Photographs for Patient Identification"
- 6.061 "Gender Affirming Treatment for Transgender and Gender Diverse Patients"
- 6.062 "Epinephrine Outside the Secure Perimeter"
- 6.063 "Hepatitis C Treatment"
- 6.065 "Pregnant Patient Care Labor and Birth"
- 6.067 "Opioid Prescription Use and Opioid Overdose Reversal with Naloxone"
- 6.068 "Random Urine Drug Screening"
- 6.069 "Medication Crush"
- 6.070 "Override Process for Medications in Emergency Situations"